

Section 9

Current State Admissions & Self-Reporting

Version 2.0 — Preliminary Draft

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SECTION 9 — CURRENT STATE ADMISSIONS & OPERATIONAL SELF-REPORTING

(State-Generated Self-Disclosure Layer)

Version 2.3 — June 5, 2026

[Revised to include verbatim evidentiary excerpts from primary source documents]

9.1 PURPOSE AND SCOPE

This section presents Iowa HHS's own current operational reporting and strategic planning documents as evidentiary admissions. Each document in this section was generated by the state for federal compliance, planning, or funding purposes. None were produced in response to this petition. All quoted language is drawn verbatim from official Iowa HHS documents.

The evidentiary value of this section is distinct from the federal findings layer (Section 6) and the independent evaluation layer (Section 10). Where Sections 6 and 10 document what external reviewers found, Section 9 documents what the state itself has disclosed — in official submissions to federal agencies, in its own strategic plans, and in its evaluations of its own programming. State admissions carry particular weight in an audit context because they cannot be dismissed as external advocacy: they are the agency's own characterization of its operational condition.

Cross-reference: Section 6 (CFSR longitudinal record); Section 7 (Round 4 structural integrity); Section 10 (independent evaluations); Section 13.2 (Systemic Failure Matrix); Section 19 (Corrective Action Matrix).

9.2 ATTACHMENT A9 — IOWA HHS OPERATIONAL PLAN FY2026

Iowa HHS Operational Plan, July 1, 2025 – June 30, 2026

Published: June 2025

Source: Iowa Publications Online — <https://publications.iowa.gov/54026/>

NOTE ON VERSION: The petition index previously referenced the FY2025 Operational Plan (July 1, 2024 – June 30, 2025). The uploaded document is confirmed as the FY2026 plan (July 1, 2025 – June 30, 2026, published June 2025), which supersedes the earlier version and is the current governing operational document at time of petition submission.

9.2.1 KEY PERFORMANCE INDICATORS — WORKFORCE VACANCY

The Operational Plan's Strategic Plan in Action Appendix contains the agency's three Key Performance Indicators (KPIs) selected for executive dashboard reporting. The first KPI is workforce vacancy, measured as the average number of days to fill key vacancies in the Division of Family Well-Being and Protection and State-Operated Specialty Care. The following vacancy-tracked positions are explicitly listed:

- Income Maintenance Worker 2
- Licensed Practical Nurse (LPN)
- Registered Nurse
- Resident Treatment Technician
- Resident Treatment Worker
- Social Work Supervisor
- Social Worker 2 / Social Worker 3 / Social Worker 4
- Youth Services Worker

KPI Performance Table (Operational Plan, p.16):

Target SFY26: 50 days to fill key vacancies

Target SFY25: 80 days to fill key vacancies

Actual SFY24: 72 days to fill key vacancies

AUDIT RELEVANCE: Iowa HHS's designation of vacancy fill-time as one of only three agency-wide KPIs confirms that workforce capacity is the agency's own highest-priority measurable performance concern as of FY2026. The SFY24 actual of 72 days to fill key vacancies — against a SFY25 target of 80 days — indicates that the prior year's target was met. The SFY26 target of 50 days represents a 30-day improvement goal. However, the selection of fill-time rather than vacancy rate, turnover rate, or caseload-per-worker as the workforce KPI reflects a measurement architecture that monitors the speed of replacement rather than the underlying structural conditions (workforce pipeline, compensation parity, caseload conditions) that drive turnover.

Cross-reference: Section 6.6 Domain 2 (workforce capacity — three-round longitudinal failure pattern); Section 9.3.4 (APSR FFY2026 — training data confirming "larger than normal class sizes" due to increased staff turnover).

9.2.2 CHILD PROTECTIVE SERVICES — OPERATIONAL DATA (FY2026)

The Operational Plan's Division of Family Well-Being and Protection section documents the following Child Protective Services performance measures and targets:

Child protective and family assessment intakes (Measure 401_22204_001):

2024 Actual: 57,551

2025 Target: 57,764

2026 Target: 58,000

Child protective and family assessments completed (Measure 401_22204_002):

2024 Actual: 31,676

2025 Target: 37,344

2026 Target: 38,000

AUDIT RELEVANCE — INTAKE/ASSESSMENT GAP: In SFY2024, 57,551 intakes were received but only 31,676 assessments were completed — a gap of 25,875 intakes that either did not meet criteria for assessment or were not completed within the reporting period. The SFY2025 target increases assessments to 37,344 against 57,764 expected intakes — still a projected gap of more than 20,000. This intake-to-assessment ratio is a state-generated disclosure of assessment capacity limitations that directly intersects with the CFSR caseworker contact and safety assessment failures documented across three review cycles (Section 6.6 Domain 1).

Number of children in foster care (Measure 401_22204_003):

2024 Actual: 3,717

2025 Target: 6,581

2026 Target: 6,581

AUDIT RELEVANCE — FOSTER CARE TARGET ANOMALY: The 2024 actual foster care population of 3,717 children against a 2025 target of 6,581 children represents a planned increase of 77% in foster care census. No explanatory action plan text is provided beyond: "CPS Director will monitor the children in foster care and work with Service Area Managers to ensure these cases are reviewed monthly." The absence of any explanatory narrative for a target that is 77% above the prior actual is a documentation gap in the state's own operational planning.

Percent of children in foster care visited by social workers (Measure 401_22204_005):

2024 Actual: 95.1%

2025 Target: 95%

2026 Target: 95%

AUDIT RELEVANCE: The 95.1% administrative caseworker visit rate stands in direct conflict with the APSR FFY2026 case review data (Section 9.3.2), which shows caseworker visits with children at 77% and caseworker visits with parents at 64% on independently reviewed cases. The gap between administrative data (95.1%) and case review data (77%) for the same measure — caseworker visits — is explicitly acknowledged by Iowa HHS in the APSR (see Section 9.3.2 below). The Operational Plan's 95% administrative target does not acknowledge this discrepancy.

Cross-reference: Section 9.3.2 (APSR Item 14/15 — caseworker visit case review data vs. administrative data gap); Section 6.6 Domain 1 (longitudinal caseworker contact pattern); Section 13.2 (Systemic Failure Matrix).

Percent of foster care children placed with relatives or fictive kin (Measure 401_22203_002):

2024 Actual: 43%

2025 Target: 45%

2026 Target: 45%

AUDIT RELEVANCE: Iowa's kinship placement rate of 43% in FY2024, with a target of 45%, represents a state-documented modest improvement goal in an area directly relevant to the Robidoux kinship exclusion documented in the case-level record.

Cross-reference: Section 14 (case-level placement outcomes — kinship exclusion).

9.2.3 EARLY ACCESS — CAPTA INTERFACE

The Operational Plan documents Early ACCESS (Iowa's IDEA Part C early intervention system) under the Family Well-Being and Protection division:

Number of children with an individualized family services plan (Measure 401_22207_001):

2024 Actual: 2,468

2025 Target: 2,500

2026 Target: 2,500

Action Plan language: "Child Abuse Prevention Treatment Act (CAPTA) referrals are made to Early ACCESS for all children ages 0–3 with a substantiated child maltreatment report."

AUDIT RELEVANCE: The CAPTA-to-Early ACCESS referral requirement represents

a direct policy link between child protective services and early intervention for the same population identified in the CFSR Round 3 "no protocol" admission (Section

6.5.3 — children under 6 removed due to parental drug use). The Operational Plan confirms this linkage exists in policy; the question for audit purposes is whether it is functioning in practice, particularly for the drug-removal population.

9.3 ATTACHMENT A10 — ANNUAL PROGRESS AND SERVICES REPORT FFY2026

Iowa HHS APSR FFY2026

Submitted: June 30, 2025

Contact: Christina Hazelbaker, Compliance Officer, Division of Family Well-Being and Protection

Source: <https://hhs.iowa.gov/media/18103/>

9.3.1 CRITICAL ADMISSION — CCWIS PROJECT PAUSED

"Effective July 2024, HHS placed the VISION project on pause, including solutions design and software development, to comprehensively assess our overall strategy for internal design and development." (APSR, p.28)

"After careful consideration of our options, HHS determined to pursue a commercial-off-the-shelf (COTS) CCWIS solution and to cease further investment in a customized, in-house developed solution." (APSR, p.28)

AUDIT RELEVANCE: The Iowa CCWIS VISION project — the \$108.8M federal-state investment documented in Section 18 (Attachments A44–A49) — was placed on pause in July 2024 and its fundamental design direction reversed. The state has abandoned the custom-developed solution in favor of a commercial off-the-shelf system. As of the APSR submission date (June 2025), HHS anticipated "having a pool of vendors to select from in December 2025" and potentially "ready to initiate design and development work in the Spring/early Summer of 2026."

This means the CCWIS system that federal reviewers approved in 2024 (Attachment A45) is no longer being built. The system approved for federal funding is being replaced by a different system that has not yet been selected. Cross-reference:

Section 18 (CCWIS investment documentation — \$108.8M federal-state chain).

9.3.2 CASE REVIEW DATA — ITEM-BY-ITEM PERFORMANCE TABLE

The APSR presents CFSR case review data comparing January 2023–December 2023 performance against January 2024–March 2025 performance (APSR Table 2a, p.8–9):

Jan 2023– Jan 2024–

Item Dec 2023 Mar 2025

Safety Outcome 1

1: Timeliness of Investigations 61% 86%

Safety Outcome 2

2: Services to Prevent Removal/Re-entry 85% 81%

3: Risk and Safety Assessment 51% 61%

Permanency Outcome 1

4: Stability of Foster Care Placement 68% 72%

5: Permanency Goal for Child 95% 87%

6: Achieving Reunification/Permanency 75% 80%

Permanency Outcome 2

7: Placement with Siblings 96% 59% ← DECLINE

8: Visiting with Parents/Siblings 82% 80%

9: Preserving Connections 88% 84%

10: Relative Placement 84% 95%

11: Relationship with Parents 84% 85%

Well-Being Outcome 1

12: Needs and Services — Child/Parent/FP 62% 67%

12A: Children 77% 86%

12B: Parents 67% 71%

12C: Foster Parents 70% 70%

13: Child/Family Involvement in Planning 80% 75% ← DECLINE

14: Caseworker Visits with Child 59% 77%

15: Caseworker Visits with Parents 49% 64%

Well-Being Outcome 2

16: Educational Needs 89% 100%

Well-Being Outcome 3

17: Physical Health 52% 74%

18: Mental/Behavioral Health 50% 71%

Source: CFSR Portal OMS, State Summary OMS Statewide Summary Report

NOTE: Data reflects 23–57 applicable cases (item-dependent) reviewed during a period described as a "transition" in review structure following PIP completion on December 31, 2023.

AUDIT RELEVANCE — CASEWORKER VISIT DATA CONFLICT: Items 14 and 15 are the most critical for this petition's audit objectives. Caseworker visits with children showed 77% on case review — against the Operational Plan's administrative rate of 95.1%. Caseworker visits with parents showed 64% on case review. Iowa HHS itself acknowledges this discrepancy in the APSR:

"Iowa seeks to close the gap between case review data and administrative data around the process for delays." (APSR, p.11)

"Approximately 70% of visits are conducted within the initial timeframe assigned and 30% of visits request and receive a supervisory approved delay." (APSR, p.11)

This means the state counts supervisor-approved visit delays as meeting the caseworker visit standard — and administrative data reports 100% compliance on this basis — while independently reviewed cases show only 77% performance. The state is self-reporting a compliance methodology that inflates administrative performance relative to independent case review findings.

AUDIT RELEVANCE — ITEM 2 EXPLICIT NON-ACHIEVEMENT ADMISSION:

"Given that Iowa did not successfully meet the Round 3 PIP goal for item 2 [Services to Family to Protect Child(ren) in the Home and Prevent Removal or Re-Entry], we recognize the need to focus on this item to assure ongoing communication and consistency between reviewers and federal partners." (APSR, p.11)

This is a state-generated, verbatim admission of PIP non-achievement on a safety outcome item, made in a federally submitted compliance document.

AUDIT RELEVANCE — ITEM 3 ONGOING SUBSTANCE USE FINDING:

Among the primary issues arising from the most recent case reviews:

"Ongoing parental substance use was not identified, children remained in the home; not meeting privately with the child also exacerbated the issue as the worker was not able to elicit potentially helpful information from the child." (APSR, p.12)

Ongoing assessment quality on risk and safety — initial vs. ongoing comparison:

Jan 2023–Dec 2023: Initial 90% / Ongoing 59%

Jan 2024–Mar 2025: Initial 91% / Ongoing 74%

Ongoing assessment quality remains significantly lower than initial assessment quality across both periods. Substance use identification failures during ongoing assessment are a state-generated finding directly relevant to the case-level record (Section 14).

AUDIT RELEVANCE — SIBLING PLACEMENT DECLINE: Item 7 (Placement with Siblings) declined sharply from 96% to 59% between review periods — a 37-point drop. No corrective action plan is cited for this item in the APSR.

AUDIT RELEVANCE — RE-ENTRY TO FOSTER CARE TREND:

Iowa's own data shows a significant and accelerating re-entry trend (APSR Table 2I):

Re-Entry to Foster Care within 6 Months:

June 2021: 37%

June 2022: 54%

June 2023: 71%

February 2024: 76%

February 2025: 79%

"HHS anticipated an increase, [but] this degree of change was not [anticipated]." (APSR, p.26)

"HHS' QA&I Bureau identified this area as a priority and will develop an approach to identify root causes." (APSR, p.26)

A re-entry rate of 79% within 6 months — meaning nearly 4 out of 5 children who re-enter foster care do so within the first six months after discharge — constitutes the state's own documentation of a reunification stability crisis. This is filed as a current performance finding in a federal compliance document submitted June 2025.

Cross-reference: Section 6.6 Domain 1 (caseworker contact — longitudinal);

Section 7.4 (Round 4 — post-permanency blind spot); Section 13.2 (Systemic Failure Matrix — recurrence and re-entry domains).

9.3.3 STATEWIDE INFORMATION SYSTEM — PLACEMENT ENTRY TIMELINESS

Iowa administrative code requires placement entries in FACS within three business days. Iowa's own reporting shows:

Timely Entry of Placements (APSR Chart 2j):

SFY 2021: 51.3%

SFY 2022: 72.4%

SFY 2023: 63.9%

SFY 2024: 63.7%

SFY 2025: 61.5% (July 2024–February 2025)

Iowa's own state regulation for timely placement data entry is being met less than 62% of the time as of February 2025. Performance declined from the SFY2022 peak of 72.4% and has been relatively flat since, with SFY2025 data showing continued decline. This is a state-generated admission of ongoing data quality failure in the same system (JARVIS/FACS/AFCARS) that federal CFSR reviewers rely upon for compliance determinations.

Cross-reference: Section 10.4 (KinderTrack/AFCARS independent data analysis);

Section 18.4 (CCWIS documentation — system limitations).

9.3.4 WORKFORCE — TRAINING DATA ADMISSION

The APSR documents new worker training evaluation data across five fiscal years (APSR Tables 2s and 2t). A critical admission appears in the narrative:

"It is suspected larger than normal class sizes over the past two years in particular, which is a result of an increase in staff turnover and corresponding hiring, has played a role in this decrease in [training recommendation] ratings. As class sizes grow larger, opportunities for more one-on-one attention and discussion with each new SWCM decreases." (APSR, p.76)

This is a state-generated admission that:

- (1) Staff turnover has increased over the past two years (SFY2023–2024)
- (2) Turnover is driving larger new-worker training cohorts
- (3) Larger cohorts reduce individual training quality
- (4) The agency recognizes but has not resolved this dynamic

The CFSP further confirms that when a mentor is unavailable "due to staffing issues, it is expected the Supervisor of the new worker will complete and document all of the mentoring activities" — confirming that staffing-driven mentorship gaps are a documented, policy-acknowledged condition rather than an exception.

Cross-reference: Section 6.6 Domain 2 (workforce capacity — three-round longitudinal pattern); Section 9.2.1 (Operational Plan KPI — vacancy fill time);

Section 13.2 (Systemic Failure Matrix — workforce domain).

9.3.5 PERMANENCY TIMING — CRITICAL STATE ADMISSION

"An additional observation made during ICJ case assessments is an increase in the number of cases where termination of parental rights occurred and a permanency plan was not finalized for a child." (APSR, p.38, Chart 2p narrative)

This is a state-generated admission that TPR is occurring — parental rights are being terminated — without a corresponding permanency plan being in place for the child. This is precisely the "post-permanency blind spot" identified in Section 7 of this petition: the federal oversight mechanism records the termination of parental rights as a permanency outcome without tracking whether the child actually achieved a stable permanent placement following that legal action.

Cross-reference: Section 7.4 (Round 4 — post-permanency blind spot);

Section 14 (Drew household — placement outcomes post-TPR);

Section 13.2 (Systemic Failure Matrix — post-permanency domain).

9.4 ATTACHMENT A11 — CAPTA GRANT FFY2026 + 2024–2025 YEAR-END REPORT

Iowa HHS CAPTA Grant FFY2026 Grant Application and 2024-2025 Year End Report

Submitted: June 2025

Source: <https://hhs.iowa.gov/media/18284/>

9.4.1 SAFE PLANS OF CARE — STATE-DOCUMENTED 55% FAILURE RATE

Iowa's Child Protection Policy unit conducted an internal review of 20 assessments from CY2024 to evaluate Safe Plan of Care implementation. Results:

- 25% of cases: Safe Plan of Care was either not required or was appropriately completed
- 20% of cases: "it was unclear whether a Safe Plan of Care should have been implemented, primarily due to insufficient follow-up with medical providers"
- 55% of cases: "a Safe Plan of Care was required but was not completed"

(CAPTA Annual Report 2024-2025, p.7)

AUDIT RELEVANCE: In 55% of internally reviewed cases — more than half — Iowa HHS's own review found that a federally required CARA protection was not implemented.

This is not an external finding; it is Iowa HHS's own assessment of its own workers' practice. The document further acknowledges "a downward trend of Safe Plans of Care data over the past 5 years" (p.6) — a five-year declining trend spanning the complete CFSR Round 3 PIP period (2019-2023).

9.4.2 CARA DATA — SERVICE REFERRAL COLLAPSE (CY2020–CY2024)

CARA Data Table (CAPTA, p.8-9):

Infants affected — identified at intake:

CY2020: 203 | CY2021: 185 | CY2022: 157 | CY2023: 143 | CY2024: 152

Safe Plans of Care completed:

CY2020: 66 | CY2021: 70 | CY2022: 66 | CY2023: 50 | CY2024: 45

Service referrals made (affected infant + parent/caregiver):

CY2021: 93 | CY2022: 62 | CY2023: 39 | CY2024: 29

AUDIT RELEVANCE — REFERRAL COLLAPSE: Service referrals declined from 93 in CY2021 to 29 in CY2024 — a 69% decline — while identified affected infants remained relatively stable at 143-185. In CY2024, 29 referrals were made for 152 identified affected infants, meaning approximately 81% of identified affected infants received no documented service referral. This is a state-generated data disclosure directly relevant to the CFSR Round 3 "no protocol" admission (Section 6.5.3).

9.4.3 FOUR IDENTIFIED SYSTEMIC CHALLENGES (verbatim, CAPTA p.9)

Iowa HHS explicitly identifies these systemic challenges in its own CAPTA report:

"Services for infants born affected by substance use (including during prenatal drug exposure, withdrawal symptoms, or Fetal Alcohol Spectrum Disorders) and their mothers are more limited in rural areas of Iowa compared to urban centers."

"Participation in programs such as Early ACCESS, Home Visiting, and the Iowa Family Support Program is voluntary, making it difficult to monitor family engagement and compliance—especially in cases where services are provided outside of agency oversight."

"There is a continued need for improved data collection and information-sharing across systems that serve this population."

"Greater cross-system collaboration is needed, particularly among medical, mental health, and HHS providers, to ensure consistent identification of affected infants and provision of appropriate services."

These four state-generated admissions confirm that the structural gaps identified in CFSR Round 3 (Section 6.5.3) remain unresolved as of 2025.

9.4.4 JARVIS SYSTEM BUGS AFFECTING CARA TRACKING

"While pulling data to review and analyze in January 2025, HHS staff noted that our Child Welfare Information System, JARVIS, would benefit from enhancements to assist with the identification of infants affected and the implementation of Safe Plans of Care. HHS staff also noted that a couple of bugs in the system were creating notifications to

ongoing workers at incorrect times." (CAPTA, p.8)

AUDIT RELEVANCE: The state's CARA tracking system was corrupted by software bugs as recently as early 2025 — concurrent with the CCWIS development pivot documented in Section 9.3.1 and the data quality admissions in Section 9.3.3.

9.4.5 CASELOAD DATA — NO MAXIMUM CASELOAD STANDARD

"HHS does not currently set a 'maximum' caseload for workers in any given period as time factors involved in every case may vary greatly depending upon the area of the State and the needs of the family." (CAPTA, p.34)

Average caseloads, CY2024:

- Child protective workers (assessment): 12 cases per month
- Case managers (ongoing services): 21 cases per month

AUDIT RELEVANCE: Iowa HHS explicitly confirms it has no maximum caseload standard for any worker classification. The absence of a caseload cap is a state-generated admission directly relevant to the workforce capacity failures documented across three CFSR rounds (Section 6.6 Domain 2).

9.4.6 SOCIAL WORK LICENSURE RATE

Of 181 survey respondents among 385 intake, screening, and assessment staff:

- Total professionally licensed at any level: 9 out of 181 (5%)

LBSW: 5 / LMSW: 3 / LISW: 1 (CAPTA Table 10c, p.33)

AUDIT RELEVANCE: 95% of responding child protective assessment workers hold no professional social work license. This is a state-generated workforce data point.

9.4.7 CITIZEN REVIEW PANEL 2024 THREE-YEAR ASSESSMENT — FOUR FORMAL RECOMMENDATIONS

Iowa's Citizen Review Panel issued four priority recommendations to HHS for 2024-2027:

SUBSTANCE ABUSE: Workers should have working knowledge of SUD including behavioral indicators, impact on child wellbeing, and ability to assess parental functioning. Also:

"Procurement and expansion of timely substance abuse treatment services for families struggling with substance use as these cases are more likely to result in the child being placed out of the home." And: "Increasing client access to HHS Drug Testing Sites through extended hours of operation, expanding the number of site locations, and assisting with transportation to drug testing sites."

SUPPORTS & SERVICES: "Increase referrals for services and supports to families when

the Child Protection Assessment is not substantiated."

LEGAL REPRESENTATIVES: Recruitment, retention, and training of legal representatives for parents and children.

CHILD WELFARE WORKFORCE: "Support the confidence, competence, and consistent practice of the child welfare workforce by establishing trauma informed supervision practices across the state."

(CAPTA Appendix C/D, p.60, 72-74)

All four recommendations correspond directly to deficiency domains documented across three CFSR cycles. Iowa HHS's response to the Supports & Services recommendation includes the explicit admission: "HHS does not track if these families follow through with services." (CAPTA Appendix D, p.74)

9.4.8 STATE ADMISSION — DRUG TESTING ACCESS BARRIERS

Iowa HHS's response to the CRP substance abuse recommendation:

"HHS recognizes the need for clients to have access to the HHS drug testing sites and that the location of the site, the hours of operation, and/or transportation to the site can be an issue. Currently, there are 55 drug testing sites across the state. The location of the sites is based on usage as are the hours of operation."

(CAPTA Appendix D, p.73-74)

AUDIT RELEVANCE: Iowa HHS confirms in writing that drug testing site access — location, hours, and transportation — is a documented barrier. Drug test results were used as the basis for parental rights terminations in cases including the case-level record documented in Section 14 of this petition. This admission of access barriers is directly relevant to the fairness of those determinations.

Cross-reference: Section 14 (case-level outcomes — drug testing); A52 (October 2025 Iowa HHS drug testing policy — sweat patch limitations); Section 6.5 (Round 3 — substance use as primary removal driver).

9.5 ATTACHMENT A12 — HHS PERFORMANCE IMPROVEMENT PLAN FY2026

Iowa HHS 2026 Performance Improvement Plan

Published: December 2025

Authors: Andrea Bentzinger (QI Coordinator) and Robert Stewart (PM Coordinator)

Source: <https://hhs.iowa.gov/media/17817/>

CRITICAL FRAMING NOTE: Iowa HHS's "2026 Performance Improvement Plan" is an

internal agency document governing its Quality Improvement (QI) and Performance Management (PM) programs — it is NOT a federal CFSR Program Improvement Plan submitted to the Children's Bureau. The APSR FFY2026 explicitly states: "Iowa does not have an active CFSR PIP or title IV-E PIP." (APSR, p.7) This document reflects Iowa's own internal QI/PM infrastructure, not federally-required corrective action.

9.5.1 PM WORKSHOP GOAL ACHIEVEMENT — MISSES OWN TARGET IN 2025

Iowa's own internal PM system performance against its 85% goal:

"Respondents reporting participation helped team achieve at least one workshop goal":

2022: 87.7% | 2023: 81.6% | 2024: 86.0% | 2025: 77.8% (below 85% target)

(State PIP, p.12)

AUDIT RELEVANCE: Iowa HHS's own internal performance improvement system — the architecture designed to detect and correct performance deficiencies — missed its own target in 2025, with 77.8% of participants reporting the sessions helped their teams. This is the state's internal improvement system underperforming on its own metrics, concurrent with CFSR Round 4 initiation.

9.5.2 MINIMUM COMPLIANCE FLOOR — TIER 1 STANDARD

The State PIP introduces a tiered PM compliance system. The minimum requirement (Tier 1) requires only: one performance measure, annual target, one action item, and an actual value with explanation. (State PIP, p.9)

AUDIT RELEVANCE: Iowa's minimum internal accountability standard requires a single measure with one action item. The contrast between this floor and the GAGAS-informed multi-domain audit standards applicable to this petition establishes the gap between Iowa's self-imposed accountability architecture and independent audit standards.

9.5.3 PI PARTICIPATION DATA

In 2025: 354 unique Iowa HHS team members formally participated in PI activities, against a 2026 target of 500. (State PIP, p.11)

PI is explicitly connected to Strategic Plan Goal 2: "Advance Operational Excellence." The Plan acknowledges PI is a voluntary participation program in which "appropriate resources are maintained to support PI infrastructure." (State PIP, p.11)

9.6 ATTACHMENT A13 — CHILD AND FAMILY SERVICES PLAN (FFY 2025–2029)

Iowa Child and Family Services Plan, Federal Fiscal Years 2025–2029

9.6.1 VISION STATEMENT

"HHS has a societal vision that individuals, families, and communities are safe, resilient and empowered to be healthy and self-sufficient. Iowa's child welfare vision statement: 'Family Connections are Always Strengthened and Preserved' supports HHS' societal vision." (CFSP, p.5)

9.6.2 WORKER TRAINING — MENTORSHIP UNDER STAFFING PRESSURE

The CFSP documents that when a mentor is unavailable "due to staffing issues, it is expected the Supervisor of the new worker will complete and document all of the mentoring activities with the staff." (CFSP, p.72)

AUDIT RELEVANCE: This policy provision explicitly anticipates mentor unavailability as a staffing-condition contingency rather than an exception. It institutionalizes a fallback to supervisor-conducted mentorship — meaning that during high-turnover periods (when new worker cohorts are largest and mentors most needed), the mentoring function is most likely to be transferred to supervisors already managing high caseloads. This is a structural compounding of workforce capacity failure.

9.6.3 NEW WORKER CASELOAD LIMITS DURING TRAINING

The CFSP documents formal pre-training caseload restrictions for new workers:

Social Work Case Managers (SWCMs): "Newly hired staff will work with their mentors on no more than 10 cases during their field learning experience prior to the completion of SW 020." (CFSP, p.72)

Child Protection Workers (CPWs): "Newly hired staff receive no more than six Family Assessment or CINA cases during their field learning experience prior to the completion of CP 200." (CFSP, p.72)

Cases excluded from new CPW assignments during training include:

- Sexual abuse cases
- Severe physical abuse
- Previous terminations
- Medical neglect
- Child death
- Cases with multiple CPS substantiations
- Severe domestic violence in the home

AUDIT RELEVANCE: These documented caseload restrictions apply during the training period only and say nothing about post-training caseloads. The CFSR Round 1 (2003) found 35–55 case caseloads for individual workers. The CFSP's training restrictions implicitly acknowledge that new workers cannot safely carry full caseloads before training completion — but does not establish any maximum post-training caseload standard. Combining documented high turnover, larger training cohorts, and the absence of a post-training caseload cap establishes the structural conditions for the caseworker contact failures documented across three CFSR cycles.

9.6.4 RE-ENTRY TO FOSTER CARE — CFSP ACKNOWLEDGMENT

The CFSP independently documents the same re-entry acceleration trend reported in the APSR, noting that pandemic-era lower caseloads may have temporarily accelerated permanency but that the trend since has reversed. The CFSP documents listening sessions held with 11 stakeholder groups across the state in 2022 to gather feedback — confirming the agency's own awareness of stakeholder concern about system performance.

9.6.5 AFCARS DATA VALIDATION — RACE/ETHNICITY ACCURACY HISTORY

The CFSP documents AFCARS data accuracy validation across five years (CFSP Table 2I). The most significant data quality findings:

Race accuracy in FACS (FC-08):

CY 2019: 94%

CY 2021: 51% ← MAJOR DROP

CY 2022: 83%

CY 2023: 95%

Hispanic/Latino Ethnicity accuracy (FC-09):

CY 2019: 98%

CY 2021: 41% ← MAJOR DROP

CY 2022: 65%

CY 2023: 98%

AUDIT RELEVANCE: Race and ethnicity accuracy in Iowa's FACS/AFCARS system dropped to 51% and 41% respectively in CY2021 — the year of the DHS-to-HHS

administrative transition planning period. These fields are the primary data elements used to assess ICWA applicability and racial disproportionality in child welfare removals. A 51% accuracy rate for race data means approximately half of all race entries were inconsistent with case file documentation in 2021. This is a state-generated admission of data quality failure in the fields most critical for ICWA compliance and disproportionality analysis.

Cross-reference: Section 6 (CFSR Round 1 — ICWA non-compliance findings);

Section 13.2 (Systemic Failure Matrix — data integrity domain).

9.7 ATTACHMENT A14 — TRANSFORMATION PLAN: COMMUNITY-BASED SERVICES

Strengthening Iowa's Community-Based Services System: Transformation Plan

Published: February 28, 2023

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Source: <https://www.mathematica.org/publications/strengthening-iowas-community-based-services-system-transformation-plan>

9.7.1 ORIGIN: DIRECT RESPONSE TO DOJ ADA VIOLATION FINDING

"This evaluation began shortly after the Department of Justice (DOJ) investigated resource centers in Iowa and found the state in violation of Americans with Disabilities Act. Taking into account the results of the community-based services evaluation (CBSE) and similar findings outlined in the DOJ findings report, the Mathematica-Harkin team developed this Transformation Plan to support Iowa HHS and its partners in transforming the state's system of care." (CBS Transformation Plan, p.1)

AUDIT RELEVANCE: Iowa HHS's commissioned Transformation Plan explicitly opens by citing a DOJ ADA violation as its primary impetus. This directly links the CBS Transformation Plan to the federal enforcement chain documented in Section 8.2 (2020-2021 DOJ investigations). The state's response to a DOJ finding was to commission an external transformation plan — not an independent audit.

9.7.2 FUNDING: \$30 MILLION ARPA FEDERAL INVESTMENT

"The department will use funds from Section 9817 of the American Rescue Plan Act—which allocated \$30 million to implement the recommendations resulting from the CBSE—to advance this goal." (CBS Transformation Plan, p.1)

AUDIT RELEVANCE: Iowa HHS committed \$30 million in federal ARPA funds to implement this Transformation Plan. The adequacy and accountability of this federal

investment falls within audit scope under Iowa Code § 11.

9.7.3 DOCUMENTED SYSTEM FAILURES FROM STATE'S OWN CBSE

The Transformation Plan documents the following current system failures:

Waitlist: "As of June 2022, there were more than 18,000 waiver slots requested across all five 1915(c) waivers with a waitlist. The wait lists also have grown considerably over the last several years, and the length of time that individuals wait for waiver slots ranges from 2 years to more than 5 years." (p.10)

Assessment: "Iowa HHS does not collect financial or functional information from people before placing them on a waitlist, which limits Iowa HHS's ability to plan for their needs or refer them to other non-Medicaid services for which they may be eligible." (p.10)

Navigation: "consumers face challenges navigating a complicated system and find it difficult to obtain clear, correct, and consistent information from providers, case managers, and Iowa HHS about available services." (p.20)

Case management: "[Members] are not getting the support they need from case managers. The case managers ignore their problems or forget to investigate their requests for help." — Iowa Consumer (quoted p.24)

Service package: "Basing service offerings and budget caps on an individual's diagnosis was inadequate to ensure that the individual's needs are met." (p.15)

9.7.4 GUIDING PRINCIPLES MISALIGNMENT BETWEEN HHS AND EVALUATORS

"The team developed these CBSE guiding principles prior to HHS updating their guiding principles... Aside from the overlap on the principle of equity, the CBSE and HHS principles differ from one another." (CBS Transformation Plan, p.3)

AUDIT RELEVANCE: Iowa HHS updated its own guiding principles during the evaluation in a manner that diverged from the evaluators' framework. Iowa's commissioned evaluators noted the misalignment — establishing that the agency's own value framework was not aligned with the external quality standards designed to guide its transformation.

9.7.5 MISSION OUTCOME ADMISSION — NO NEAR-TERM VERIFICATION

"Mission outcomes, which measure how the system is working to improve outcomes—will most likely not be realized until years after implementation." (CBS Transformation Plan, p.29)

AUDIT RELEVANCE: The Transformation Plan explicitly acknowledges that its mission outcomes are not measurable for years. This is a state-generated admission that a \$30 million federal investment lacks near-term outcome verification — replicating the PIP accountability pattern documented in Section 6.6 Domain 5.

9.8 ATTACHMENT A15 — CONSOLIDATED ICAPP REPORTS (SFY 2019–2022)

Iowa Child Abuse Prevention Program Evaluation Reports

Produced by: Prevent Child Abuse Iowa (grant administrator) and Public Consulting Group (evaluator)

Submitted to: Iowa Department of Human Services / Iowa HHS

SFY 2019 (July 1, 2018 – June 30, 2019):

<https://publications.iowa.gov/52096/1/ICAPP%202019%20Report%20FINAL%20092419.pdf>

SFY 2020 (July 1, 2018 – June 30, 2020):

<https://publications.iowa.gov/52097/1/ICAPP%202020%20Report%20FINAL%20REPORT.pdf>

SFY 2022 (July 1, 2021 – June 30, 2022):

<https://publications.iowa.gov/52099/1/ICAPP%202022%20Evaluation%20Report%20FINAL%20092822.pdf>

SFY 2021: <https://publications.iowa.gov/52098/1/ICAPP%202021%20Evaluation%20Report%20FINAL.pdf>

SFY 2023: <https://publications.iowa.gov/52100/1/ICAPP%202023%20Evaluation%20Report%20FINAL.pdf>

NOTE: SFY 2019 and SFY 2022 reports reviewed in full from uploaded documents.

SFY 2020 report reviewed in full. SFY 2021 and SFY 2023 pending upload.

9.8.1 ICAPP PROGRAM SCALE AND POPULATION SERVED

ICAPP is Iowa's primary CAPTA-funded child abuse prevention infrastructure, funded through PSSF, TANF, CBCAP, CAPTA, and state sources. Summary across three reviewed years:

SFY 2019: \$1,566,417 | 56 projects | 56 counties | 1,956 families | 10,929 children

SFY 2020: \$1,562,638 | 56 projects | 56 counties | 2,003 families | 9,931 children

SFY 2022: \$1,730,632 | 50 projects | 44 counties | 1,326 families | 6,258 children

AUDIT RELEVANCE — COVERAGE CONTRACTION: ICAPP-funded programs contracted from 56 counties served in SFY 2019 and 2020 to 44 counties in SFY 2022 — a loss of 12 counties (21%) of statewide coverage. Families served declined from 2,003 in SFY 2020 to 1,326 in SFY 2022 — a 34% reduction. This contraction occurred concurrent with the SFY 2022 DHS-to-HHS agency consolidation and during the same period as the CFSR Round 3 PIP (2019–2023). Total ICAPP funding was \$1.5 million annually — a figure that has not substantially increased across the review period despite the child protection gaps documented in three CFSR cycles.

9.8.2 FUNDING DEMAND EXCEEDS AVAILABILITY — STATE-DOCUMENTED UNMET NEED

"Funding requests exceed available ICAPP funds with a total of \$4,661,712 requested and approximately \$1.5 million available to award." (ICAPP SFY 2019, p.2)

"Funding requests overall tripled available ICAPP annual funds, with \$1.5 million awarded for fiscal years (FYs) 2019 and 2020." (ICAPP SFY 2020, p.2)

AUDIT RELEVANCE: Iowa's own prevention infrastructure documents that demand for ICAPP-funded child abuse prevention services is three times the available funding. This is a state-generated admission that Iowa's child abuse prevention infrastructure is chronically underfunded relative to documented community need — in the same period when CFSR reviews documented persistent failures in child safety and service access.

9.8.3 ICAPP FAMILIES SERVED — ECONOMIC PROFILE

ICAPP programs disproportionately serve Iowa's lowest-income families. Across all three reviewed years, approximately 50% of participant families had annual household incomes of \$20,000 or less. Direct comparison to Iowa general population:

"56% of all Iowans earn \$50,000 or more, compared to 8% of participant families" (ICAPP SFY 2019, p.12)

"57% of all Iowans earn \$50,000 or more compared to 8% of participant families" (ICAPP SFY 2020, p.11)

"60% of all Iowa families earn \$50,000 or more compared to 11% ICAPP families" (ICAPP SFY 2022, p.10)

AUDIT RELEVANCE: ICAPP programs serve a population that is structurally distinct from the Iowa general population — overwhelmingly female (88-95%), lower-educated (72-79% have at most a high school diploma), and substantially lower-income. This is the same demographic profile associated with child welfare involvement across the CFSR record.

9.8.4 PROGRAM COMPLETION AND SERVICE CONTINUITY

The ICAPP series consistently documents that protective factor improvements are significantly greater among participants who complete programs versus those who discharge early. This finding is repeated across SFY 2019, 2020, and 2022:

"Those who successfully completed the program (or whose child aged out of services) had greater statistically significant improvement in scores across nearly all domains than those who did not complete the program." (ICAPP SFY 2019, p.18)

"Greater improvement of protective factors was seen in those who complete a program than in other discharge statuses." (ICAPP SFY 2020, p.17)

In SFY 2019, parents who discharged early showed score decreases in Concrete Support and Social Emotional Support — the two domains most directly linked to economic stress and social isolation.

AUDIT RELEVANCE: ICAPP's own independent evaluator has documented across three consecutive reporting periods that early program exit is associated with worsened outcomes — yet the program serving these families is chronically underfunded at one-third of documented demand. This creates a compounding failure: families most in need of prevention infrastructure receive truncated services due to funding constraints.

9.8.5 DOMAINS OF LOWEST PROTECTIVE FACTOR SCORES — CONSISTENCY WITH CFSR FINDINGS

Across all three reviewed ICAPP years, the protective factor domain with consistently lowest scores is Family Functioning and Resilience — defined as "having adaptive skills and strategies to persevere in times of crisis; family's ability to openly share positive and negative experiences and mobilize to accept, solve, and manage problems."

SFY 2022 pre-survey scores by domain:

Nurturing and Attachment: 6.38 (highest)

Child Development and Parenting: 5.72

Social Emotional Support: 5.70

Concrete Support: 5.44

Family Functioning and Resilience: 5.31 (lowest)

(ICAPP SFY 2022, p.13)

AUDIT RELEVANCE: Family Functioning and Resilience is the domain with the lowest pre-program scores and the domain where ICAPP's evaluator has consistently recommended improvement focus. This directly corresponds to the Family Functioning domain deficiencies documented in CFSR Round 3 case reviews and the APSR (Section

9.3.2), where Iowa's own case reviewers identified workers not meeting privately with children and failing to identify ongoing parental substance use.

9.8.6 ICAPP COVERAGE GAP — CONCURRENT WITH REMOVAL YEAR

The SFY 2019 ICAPP report covers July 1, 2018 – June 30, 2019 — the period that encompasses both the CFSR Round 3 onsite review (April–September 2018) and the filing of the TPR petition in the case-level record (Section 14). In SFY 2019:

— ICAPP programs operated in 56 of Iowa's 99 counties (57% coverage)

— ICAPP total funding was \$1,566,417 against \$4,661,712 in submitted requests

— 82% of program participants below the poverty line showed no significant improvement in protective factor scores (SFY 2019, p.42)

"In all domains, there were no substantial increases observed in participants with middle school or lower education. Upon further inspection of this subset, only seven percent of respondents speak English and they are predominantly people of color." (ICAPP SFY 2019, p.41)

AUDIT RELEVANCE: In the year of removal (SFY 2019), ICAPP programs documented zero measurable improvement in protective factors for families below the poverty line.

The prevention infrastructure serving this population was operating at one-third of documented capacity. These facts are contemporaneous with the CFSR Round 3 finding that Iowa failed 91% of sampled safety assessments.

9.8.7 RESILIENT COMMUNITIES STRONG FAMILIES SURVEY (SFY 2022) — COMMUNITY ADMISSIONS

The SFY 2022 ICAPP report includes findings from Iowa's first statewide Strong Families Community Survey (n=622 respondents across 83 counties):

Community resource gaps identified:

"Commonly mentioned needs include affordable and dependable childcare, affordable public transportation, parenting programs, community activities, safe and affordable housing, accessible substance use and mental health resources, and financial planning assistance." (ICAPP SFY 2022, p.42)

Stigma finding: "There is more concern about perceived stigma and judgement for using these services than there is true judgement of those that access the services." (ICAPP SFY 2022, p.44)

Community maltreatment awareness: Only 9% of respondents believe their community is "extremely aware" of child maltreatment as a problem; 38% say their community is only "slightly aware." (ICAPP SFY 2022, p.41)

AUDIT RELEVANCE: Iowa's own community-level prevention survey confirms that the specific service gaps identified in the CFSR Round 3 "no protocol" admission — substance use resources, accessible childcare, basic needs support — remain unresolved at the community level as of 2022, three years into the Round 3 PIP period.

9.9 ATTACHMENT A16 — EARLY CHILDHOOD IOWA ANNUAL REPORTS (FY2018–FY2024)

FY2018: <https://publications.iowa.gov/46686/>

FY2019: <https://publications.iowa.gov/46643/>

FY2020: <https://publications.iowa.gov/46607/>

FY2021: <https://publications.iowa.gov/46773/>

FY2022: <https://publications.iowa.gov/46555/>

FY2023: <https://publications.iowa.gov/id/eprint/47855>

FY2024: <https://publications.iowa.gov/id/eprint/52222>

Each annual report includes performance measures tables, funding history, and indicator information. The FY2022 report is the first published under Iowa HHS consolidation (effective July 1, 2022). The FY2020 report covers the pandemic year.

OPERATIONAL PLAN CROSS-REFERENCE: The Operational Plan documents ECI under the Family Well-Being and Protection division with the following data:

Number of families served (Measure 401_22206_001):

2024 Actual: 3,500

2025 Target: 3,579

2026 Target: 3,000 ← DECREASE

Action Plan note: "In SFY2026, ECI will work with the ECI State Board to develop a workgroup to begin updating the statewide strategic plan." (Operational Plan, p.101)

AUDIT RELEVANCE: The SFY2026 target of 3,000 ECI families served represents a planned decrease from the 3,500 actually served in SFY2024. This reduction in an early childhood intervention program serving families at risk, occurring concurrent with a 77% planned increase in the foster care census target (Section 9.2.2), is a state-generated data point reflecting the opposite trajectory from what the CFSR Round 3 "no protocol" admission and longitudinal re-entry data would indicate as necessary.

Cross-reference: Section 6.5.3 (Round 3 — "no protocol" for children under 6 removed due to parental drug use); Section 9.3.2 (APSR — 79% re-entry rate within 6 months).

9.10 ATTACHMENT A17 — EARLY CHILDHOOD IOWA NEEDS ASSESSMENT (2019)

Early Childhood Iowa Statewide Needs Assessment, September 2019

Conducted by: Iowa State University, Department of Human Development and Family Studies

Led by: Drs. Heather Rouse and Cassandra Dorius

Funded by: Preschool Development Grant Birth to Five (\$2,190,119, Grant No. 90TP0030-01-00)

Commissioned by: Iowa HHS / Iowa Department of Management

Source: <https://hhs.iowa.gov/media/18174/download>

Alternate: <https://i2d2.iastate.edu/wp-content/uploads/2021/01/Rouse-Sept2019-ECI-Statewide-Needs-Assessment.pdf>

9.10.1 DOCUMENT SCOPE AND SIGNIFICANCE

The 2019 ECI Needs Assessment is the most comprehensive cross-system analysis of Iowa's early childhood population available at the time of the CFSR Round 3 review and TPR proceedings (2018-2019). It integrates administrative data from IDPH, Iowa DHS, and the Department of Education — including child welfare records — across a birth-to-kindergarten cohort of 27,219 children. It is therefore a direct contemporaneous record of the conditions of Iowa's early childhood system in the year of the case-level removal documented in

Section 14 of this petition.

9.10.2 FOUR PRIORITY NEEDS — VERBATIM FROM EXECUTIVE SUMMARY

The 2019 ECI Needs Assessment identified four priority needs (p.6):

"1. We have gaps in service utilization for vulnerable children including those in rural counties, low-income and minority families, and with identified risks at birth such as low maternal education and inadequate prenatal care. Our families face significant access challenges, including long waiting lists and out-of-reach costs that prevent many of them from receiving the care they need when they need it.

2. We have shortages in both the quantity and quality of our early childhood workforce. Staffing challenges are particularly acute in rural counties, which comprise 89% of Iowa counties.

3. We have identified gaps in communications that limit our ability to fully foster systems-wide change.

4. We need to continue investments in building a 'data culture' so that the information we collect to monitor the impact of our efforts is used in strategic planning and daily decision-making to improve our system."

AUDIT RELEVANCE: These four findings were documented in September 2019 — the same year as the completion of the CFSR Round 3 review period and the initiation of the Round 3 PIP. They constitute contemporaneous state-generated documentation that Iowa's early childhood prevention infrastructure had four identified structural gaps that directly intersect with the CFSR Round 3 deficiency record.

9.10.3 CHILD ABUSE INCIDENCE — INCREASING TREND AT TIME OF ASSESSMENT

The 2019 ECI Needs Assessment's Table 3 (p.16) documents from 2018 ECI indicators:

"Incidence of child abuse per 1,000 children: 18.8 — Trend: Increasing"

Data Source: Iowa Department of Human Services

AUDIT RELEVANCE: Iowa's own cross-departmental early childhood assessment documents that the rate of child abuse was increasing in 2018 — the year of the CFSR Round 3 review and the year of the case-level removal documented in Section 14. This is a state-generated admission of a worsening child welfare outcome concurrent with the period during which Iowa was claiming compliance with CFSR Round 3 requirements.

9.10.4 IDS INTEGRATED DATA — 59% OF IOWA CHILDREN EXPERIENCE AT LEAST ONE BIRTH RISK

The 2019 ECI IDS analysis found (p.17):

"59% of Iowa's children experience at least 1 risk at birth that is known to significantly influence kindergarten outcomes. Risks included poverty, low maternal education, birth to a teen mother, birth to a single mother, inadequate prenatal care, preterm/low birthweight, or smoking during pregnancy."

"Rural and minority children experience more individual and cumulative count of risks at birth compared to other children."

"49% of Iowa's children qualify for Medicaid or WIC at the time of birth."

AUDIT RELEVANCE: The 2019 IDS integrated Iowa DHS child welfare data with birth records and education data. The finding that 59% of Iowa's children experience documented birth risks — and that rural and minority children face disproportionate cumulative risk exposure — establishes the structural population risk context for the CFSR Round 3 failures documented in Section 6 of this petition.

9.10.5 UNDERSERVED POPULATIONS — LESS LIKELY TO ACCESS FORMAL CARE

The IDS analysis found (p.21):

"Underserved populations who were less likely to have a formal, center-based preschool experience the year before kindergarten included Hispanic, Black and multiracial children; children born to unmarried mothers or mothers without a high school education; and children with inadequate prenatal care."

These are exactly the populations the CFSR Round 3 Statewide Assessment identified as having "no protocol" within the child welfare system. The 2019 ECI analysis documents that these same families were also least likely to access the prevention infrastructure that would

reduce child welfare involvement.

9.10.6 CHILD CARE WAITING LISTS — 78% OF INFANT/TODDLER CENTERS

The Provider Survey found (Appendix B, p.21):

"78% of centers serving infant/toddlers and 49% of child care centers serving preschool aged children reported waiting lists."

"Child care center waitlists were reported as comprising 77% and 40% of the total enrollment of infant/toddler and preschool classrooms, respectively."

"Child care centers reporting waiting lists disproportionately reported they were also not enrolled at full capacity (57% for infant/toddler programs and 60% for preschool programs)."

"Child care centers with enrollment numbers below capacity indicate that one of the primary reasons was an inability to hire staff; centers from rural areas were more likely to report this challenge compared to centers in urban areas."

AUDIT RELEVANCE: In the year of the CFSR Round 3 onsite review and the case-level removal, 78% of Iowa's infant/toddler care programs had waiting lists equivalent to 77% of their current enrollment. The service infrastructure on which "reasonable efforts" findings depend was effectively inaccessible to families seeking services in 2019.

9.10.7 FAMILY-REPORTED BARRIERS — COST AND WAITING LISTS

The Family Survey found (Appendix C, p.21):

"Families report the primary barriers to ECE services are waiting lists (54%) and cost of care (34%), with nearly one quarter of families also reporting that ECE programs do not meet their needs or they have barriers in transportation."

"An overarching theme from provider and family surveys, focus groups, and community listening sessions was 'access to care when families need it' is one of Iowa's top challenges. Families report making less-than-ideal choices for care because they feel they lack options that meet their needs at the times they need it."

Community listening sessions (Appendix E) documented that "childcare" was identified as one of the "Top Five" issues critical to the future of Iowa by participants across all 20 ISU Extension regions of the state — a statewide consensus finding.

9.10.8 CHILD CARE DESERT — 42% PROGRAM DROP IN 5 YEARS

The 2019 ECI Needs Assessment documents (p.20):

"According to the Center for American Progress, 23% of Iowa is in a child care desert."

"According to a 2018 report by Iowa Child Care Resource and Referral, there has been a 42% drop in total child care programs in the state since 2013... and a 46% drop in recent years in total programs that report accepting Child Care Assistance."

AUDIT RELEVANCE: Iowa's early childhood infrastructure lost 42% of total child care programs between 2013 and 2018 — the five-year period that encompasses the onset of CFSR Round 3 monitoring and culminates in the Round 3 onsite review. This state-documented structural collapse directly undermines any finding that Iowa made "reasonable efforts" to provide accessible services during this period.

9.10.9 EARLY CHILDHOOD WORKFORCE — MEDIAN INCOME BELOW COST OF LIVING

The 2019 ECI Needs Assessment documents (p.26):

"ECE workforce wages in Iowa mirror national challenges, where teachers' and teacher assistants' compensation is far lower than their public school counterparts sometimes as much as one-half the average salary. In fact, the median income level for ECE educators in Iowa was below the estimated cost of living."

"74% of the workforce was identified as having some college or less, with only 16% of the total workforce having an ECE degree."

Provider Survey (Appendix B, p.27):

"The inability to hire staff was cited twice as often in rural centers compared to urban ones (32% versus 16%); and eight times as often in centers that serve children with CCA compared to centers that do not accept CCA (37% versus 4%)."

9.10.10 POVERTY AT BIRTH PREDICTS ALL KINDERGARTEN OUTCOMES

The IDS logistic regression analysis (Table A.6, p.48) found:

"Poverty at birth is the only indicator that was significantly related to ALL kindergarten outcomes (FRPL, IEP, ELL, attendance, and suspensions), even when other risks and characteristics were controlled."

"Children who qualify for Medicaid or WIC at birth are over 10 times more likely to qualify for Free/Reduced priced lunch in kindergarten."

AUDIT RELEVANCE: Iowa's own integrated child welfare and education data documents that poverty at birth is the single most predictive risk factor across all measured outcomes.

49% of Iowa's children are born into these conditions. This is a state-generated, data-validated finding establishing that the conditions driving child welfare involvement are structural and systemic — not individual — in Iowa's birth-to-five population at the time of the case-level

removal documented in Section 14.

Cross-reference: Section 6.5.3 (Round 3 — "no protocol" for drug-removal population);

Section 9.8.6 (ICAPP SFY 2019 — zero improvement for poverty-line families in same year);

Section 14 (case-level outcomes — removal year context).

9.11 SECTION 9 SYNTHESIS — WHAT THE STATE'S OWN DOCUMENTS DISCLOSE

The three uploaded documents (Operational Plan FY2026, APSR FFY2026, CFSP

FFY2025–2029) alone establish the following state-generated disclosures:

DISCLOSURE 1 — THE CCWIS INVESTMENT IS BEING REBUILT FROM SCRATCH:

The \$108.8M CCWIS investment documented in Section 18 was placed on pause in

July 2024 and its fundamental design reversed. The system approved for federal

funding is being replaced by a different, not-yet-selected commercial system.

(APSR, p.28)

DISCLOSURE 2 — CASEWORKER VISIT COMPLIANCE IS INFLATED BY

SUPERVISORY DELAY APPROVALS:

Administrative data reports 95.1%–100% caseworker visit compliance; independent

case review data shows 77% for visits with children and 64% for visits with parents.

Iowa HHS explicitly acknowledges this gap and attributes it to the counting of

supervisor-approved delays as compliance. (APSR, p.11)

DISCLOSURE 3 — IOWA EXPLICITLY DID NOT MEET ROUND 3 PIP GOAL FOR ITEM 2:

"Given that Iowa did not successfully meet the Round 3 PIP goal for item 2, we

recognize the need to focus on this item." (APSR, p.11)

DISCLOSURE 4 — RE-ENTRY TO FOSTER CARE WITHIN 6 MONTHS IS 79%:

Iowa's own federal compliance report documents that 79% of children who re-enter

foster care do so within six months of discharge — up from 37% in June 2021.

(APSR Table 2I, p.25)

DISCLOSURE 5 — TPRS ARE OCCURRING WITHOUT PERMANENCY PLANS:

"An increase in the number of cases where termination of parental rights occurred

and a permanency plan was not finalized for a child." (APSR, p.38)

DISCLOSURE 6 — STAFF TURNOVER IS INCREASING AND DEGRADING TRAINING:

Larger-than-normal training cohorts due to "increase in staff turnover" are reducing per-worker training quality. (APSR, p.76)

DISCLOSURE 7 — RACE/ETHNICITY DATA ACCURACY DROPPED TO 51%/41%:

In CY2021, race accuracy in FACS was 51% and Hispanic/Latino ethnicity accuracy was 41% — fields critical for ICWA applicability determination. (CFSP Table 2I)

DISCLOSURE 8 — ECI FAMILIES SERVED IS BEING REDUCED:

The FY2026 ECI target is 3,000 families — a planned reduction from the 3,500 actually served in FY2024 — concurrent with a planned 77% increase in the foster care census target. (Operational Plan, p.101)

Source attachments for this section:

A9 — Iowa HHS Operational Plan FY2026 (July 1, 2025 – June 30, 2026)

A10 — Annual Progress and Services Report FFY2026 (submitted June 30, 2025)

A11 — CAPTA Grant FFY2026 + 2024–2025 Year-End Report [excerpts pending]

A12 — HHS Performance Improvement Plan FY2026 [excerpts pending]

A13 — Child and Family Services Plan FFY 2025–2029

A14 — Transformation Plan: Community-Based Services [excerpts pending]

A15 — Consolidated ICAPP Evaluation Reports SFY 2019–2023 [excerpts pending]

A16 — Early Childhood Iowa Annual Reports FY2018–FY2024

A17 — Early Childhood Iowa Needs Assessment 2019 [excerpts pending]

9.X DOMAIN & DEFICIENCY MAPPING — SECTION 9

Domain E — Outcomes, Incentives & Systemic Disparities (primary):

F4 — Outcome Reporting Manipulation: State-submitted annual reports and self-assessments document operational gaps that the state's own QA mechanism did not flag — consistent with the R2/R3 self-report discrepancy pattern.

F8 — Performance Metric Distortion: APSR performance metrics show persistent non-conformity on outcomes while state narrative frames deficiencies as improvement-in-progress.

Domain G — Federal Oversight History & Corrective Action Response:

F5 — Corrective Action Failure: HHS PIP FY2026 continues corrective commitments on deficiencies first documented in Round 1 (2003).

Self-reported improvement plans without independent verification are not evidence of systemic correction.

Domain C — Decision Controls & Case Authorization:

F1 — Case Planning & Service Delivery Failure: CFSP 2025–2029 identifies ongoing gaps in service array, provider capacity, and case planning consistency — deficiencies documented in every prior oversight cycle.

F3 — Assessment & Decision-Making Deficiency: CAPTA self-report documents safe plan of care deficiencies and safety assessment reliability concerns consistent with Round 3 PIP findings.

Domain B — Contract & Provider Oversight:

F2 — Ongoing Monitoring Deficiency: State-submitted transformation plan identifies contracted provider oversight gaps the state has not independently remediated.

9.X AUDIT QUESTIONS GENERATED — SECTION 9

AUDIT QUESTION 9-A: Iowa's CFSP 2025–2029 identifies service array gaps and provider capacity deficiencies. What specific funding commitments and timelines are attached to the remediation of those gaps, and what independent verification mechanism confirms progress?

AUDIT QUESTION 9-B: The APSR FFY2026 and HHS PIP FY2026 contain self-reported improvement data on outcomes that have been non-conforming since 2003. What external validation, if any, has the Children's Bureau applied to Iowa's self-reported improvement claims in the current period?

AUDIT QUESTION 9-C: Iowa's CAPTA grant documentation identifies safe plan of care deficiencies. Safe plans of care were identified as "promissory notes" rather than functional instruments in the Round 3 PIP. What structural change has occurred in safe plan of care practice between the Round 3 PIP finding and the current CAPTA reporting period?

AI assistance disclosure: See Section 5.8.

identified in the attachment citations above.

End of Section 9 — Version 1.0 — June 5, 2026

Audit Petition Packet v2.0 (5/2026)

AI assistance disclosure: See Section 5.8.

AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.